Chapter 7 The Ear and Vestibular Examination

Examination of the dizzy patient begins with observation of gait and the eyes at rest. Look for spontaneous nystagmus in primary gaze and on lateral gaze before any provocative maneuver is performed.

The Dix-Hallpike Maneuver

Explain the test to the patient first, warning that it may briefly provoke dizziness. Ask about neck and back problems, because the maneuver requires neck extension and rotation.

Seat the patient on the examination couch so that, when lying back, the head will extend over the end of the couch. Turn the head 45 degrees toward the side being tested. Holding the head firmly, lower the patient quickly into the supine position with the neck extended about 20 degrees below the horizontal. Keep the eyes open and watch them closely for at least thirty seconds.

A positive test produces vertigo together with torsional, upbeating nystagmus after a short latency. Return the patient slowly to the sitting position, watch for reversal of the nystagmus, and then repeat the test on the other side.

The test works because placing the head in this position brings the posterior semicircular canal of the lower ear into the vertical plane, so that any loose otoconia fall under gravity and stimulate the canal.

The Head Impulse Test

Ask the patient to fix their gaze on your nose. Hold the head and turn it rapidly through about 15 degrees to one side. A normal vestibulo-ocular reflex keeps the eyes on target. A corrective saccade back to the target indicates a peripheral vestibular lesion on the side toward which the head was turned.

Chapter 9 The Cardiovascular Examination

Begin with the general inspection, the hands and the pulse. Measure the jugular venous pressure with the patient reclining at 45 degrees and the head turned slightly to the left.

Palpate the apex beat and then auscultate in the four classic areas with the diaphragm and bell. A third heart sound is a low-pitched sound heard early in diastole and in adults usually indicates ventricular failure.